Thyrocare

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**Name** : Mr Sam Daniel(24Y/M)

**Date** : 29 Sep, 2026

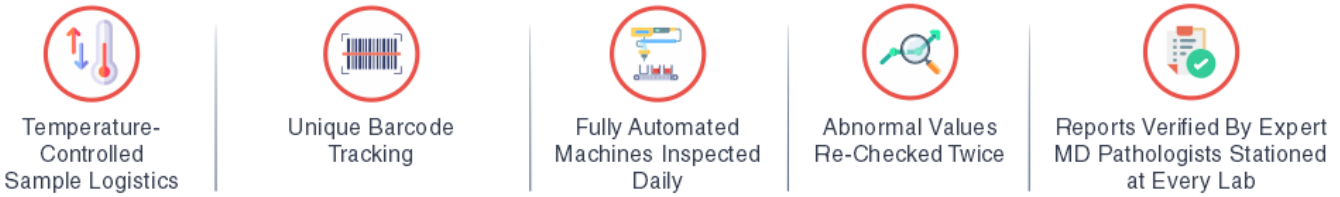
**Test Asked** : Jaanch - Mens Hairfall Screening Advanced

**Report Status** : Complete Report









**Processed At :**
UR House, No. 1056C, Avinashi Road, Coimbatore - 641018

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**Thyrocare Technologies Limited**, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703. 9870666333 wellness@thyrocare.com

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| | | | |
|---|---|---|---|
| Patient Name | : **MR SAM DANIEL (24Y/M)** | Tests Done | : **JAANCH - MENS HAIRFALL SCREENING ADVANCED** |
| Referred By | : **SELF** | | |
| Address | : **INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007** | | |

## Report Availability Summary

**Note:** Please refer to the table below for status of your tests.

**13** Ready | **0** Ready with Cancellation | **0** Processing | **1** Cancelled in Lab

| TEST DETAILS | REPORT STATUS |
|---|---|
| **JAANCH - MENS HAIRFALL SCREENING ADVANCED** | Ready with Cancellation |
| CALCIUM | Ready |
| CORTISOL | Ready |
| C-REACTIVE PROTEIN (CRP) | Ready |
| FERRITIN | Ready |
| MAGNESIUM | Ready |
| PHOSPHOROUS | Ready |
| SERUM ZINC | Ready |
| SEX HORMONE BINDING GLOBULIN (SHBG) | Ready |
| HEMOGRAM - 6 PART (DIFF) | Cancelled in Lab |
| IRON DEFICIENCY PROFILE | Ready |
| T3-T4-USTSH | Ready |
| VITAMIN D TOTAL AND B12 COMBO | Ready |
| ANTI NUCLEAR ANTIBODIES (ANA) | Ready |
| DIHYDROTESTOSTERONE (DHT) | Ready |

| | | | |
|---|---|---|---|
| Patient Name | : **MR SAM DANIEL (24Y/M)** | Tests Done | : **JAANCH - MENS HAIRFALL SCREENING ADVANCED** |
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## Tests Outside Reference Range

**Note:** Please refer to the table below for tests outside reference range.

| Test Name | Observed Value | Units | Bio. Ref. Interval. |
|---|---|---|---|
| **ELEMENTS** | | | |
| SERUM ZINC | **155** | µg/dL | 70-115 |
| **IRON DEFICIENCY** | | | |
| % TRANSFERRIN SATURATION | **65.71** | % | 13 - 45 |
| IRON | **215.54** | µg/dL | 65 - 175 |
| UNSAT.IRON-BINDING CAPACITY(UIBC) | **112.46** | µg/dL | 162 - 368 |
| **VITAMIN** | | | |
| 25-OH VITAMIN D (TOTAL) | **10.19** | ng/mL | 30-100 |

**Disclaimer:** The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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Introducing

## Health Trends

All your health reports, in one place — watch how things change over time.

OTP: 5355

CLICK or SCAN

to access detailed trends

| Test Name | Test Result & Bio. Ref. Interval |
|---|---|
| **Thyroid Function** | **29 Sep '26** |
| TSH - ULTRASENSITIVE | 0.886 µIU/mL<br>0.55 - 4.78 |
| TOTAL TRIIODOTHYRONINE (T3) | 77 ng/dL<br>60 - 200 |
| TOTAL THYROXINE (T4) | 10.4 µg/dL<br>4.5 - 12 |
| **Blood Health & Iron Status** | **29 Sep '26** |
| **IRON** | **215.54 µg/dL**<br>65 - 175 |
| **% TRANSFERRIN SATURATION** | **65.71 %**<br>13 - 45 |
| FERRITIN | 105.3 ng/mL<br>21.81 - 274.66 |
| **Vitamins, Electrolytes & Minerals** | **29 Sep '26** |
| VITAMIN B-12 | 280 pg/mL<br>211 - 911 |
| **25-OH VITAMIN D (TOTAL)** | **10.19 ng/mL**<br>30 - 100 |
| CALCIUM | 9.29 mg/dL<br>8.8 - 10.6 |
| MAGNESIUM | 2.4 mg/dL<br>1.9 - 3.1 |

**Trends Unlock After Your Next Test**

That's all for now!

Regular testing with your trusted partner helps reveal how your health is changing over time.

| | | | |
|---|---|---|---|
| **Patient Name** | : MR SAM DANIEL (24Y/M) | **Sample Collected on (SCT)** | : 29 Sep 2026 10:57 |
| **Referred By** | : SELF | **Sample Received on (SRT)** | : 29 Sep 2026 12:22 |
| **Address** | : INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007 | **Report Released on (RRT)** | : 30 Sep 2026 13:43 |
| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **25-OH VITAMIN D (TOTAL)** | **C.L.I.A** | **10.19** | **ng/mL** |

**Bio. Ref. Interval. :**
DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266–81.

**Method :** Fully Automated Chemi Luminescent Immuno Assay

| | | | |
|---|---|---|---|
| VITAMIN B-12 | C.L.I.A | 280 | pg/mL |

**Bio. Ref. Interval. :**

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

**Method :** COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

**Please correlate with clinical conditions.**

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Dr.Vinitha Jose
MD(Path)

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| | | **Sample Type \| Barcode** | : **SERUM \| FN883083** |

| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval. |
|---|---|---|---|---|
| TOTAL TRIIODOTHYRONINE (T3) | C.L.I.A | 77 | ng/dL | 60-200 |
| TOTAL THYROXINE (T4) | C.L.I.A | 10.4 | µg/dL | 4.5-12 |
| TSH - ULTRASENSITIVE | C.L.I.A | 0.886 | µIU/mL | 0.55-4.78 |

**Comments :** ***

**The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.**

**Method :**

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
USTSH - Third Generation Ultrasensitive Chemi Luminescent Immuno Assay

**Disclaimer :**Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Dr.Vinitha Jose
MD(Path)

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| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **IRON** | **PHOTOMETRY** | **215.54** | **µg/dL** |
| **Bio. Ref. Interval. :** Male : 65 - 175 Female : 50 - 170 | | | |
| **Method :** Ferrozine method without deproteinization | | | |
| TOTAL IRON BINDING CAPACITY (TIBC) | PHOTOMETRY | 328 | µg/dL |
| **Bio. Ref. Interval. :** Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl | | | |
| **Method :** Spectrophotometric Assay | | | |
| **% TRANSFERRIN SATURATION** | **CALCULATED** | **65.71** | **%** |
| **Bio. Ref. Interval. :** 13 - 45 | | | |
| **Method :** Derived from IRON and TIBC values | | | |
| **UNSAT.IRON-BINDING CAPACITY(UIBC)** | **PHOTOMETRY** | **112.46** | **µg/dL** |
| **Bio. Ref. Interval. :** 162 - 368 | | | |
| **Method :** SPECTROPHOTOMETRIC ASSAY | | | |
| FERRITIN | C.M.I.A | 105.3 | ng/mL |
| **Bio. Ref. Interval. :** Men: 21.81 - 274.66 ng/ml Women: 4.63 - 204.00 ng/ml | | | |
| **Method :** Fully Automated Chemi Luminescent Microparticle Immunoassay | | | |

**Please correlate with clinical conditions.**

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Dr.Vinitha Jose
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| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| SEX HORMONE BINDING GLOBULIN (SHBG) | C.L.I.A | 35.8 | nmol/L |

**Bio. Ref. Interval. :-**

Males 10 - 57 nmol/L

Females
Non-Pregnant : 18 - 144

Clinical Significance:

Sex hormone binding globulin (SHBG) has a high affinity for testosterone and Estradiol, and is a major factor regulating their distribution between the protein-bound and free states. The ratio of testosterone to SHBG is also known as the free androgen index (FAI) or the free testosterone index (FTI). This ratio correlates well with both measured and calculated values of free testosterone and helps to discriminate subjects with excessive androgen activity from normal individuals. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications:

Precision: Intra assay (%CV): 5.30 %, Inter assay (%CV):6.60%
Sensitivity: < 0.02 nmol/l and Specificity: no detectable cross- reactivity

Kit validation reference:

Bond A, Davis C. Sex Hormone binding globulin in clinical perspective, ActaObset Gynecol Scand 1987;66:255-62

**Please correlate with clinical conditions.**
**Method:-** SOLID-PHASE TWO-SITE CHEMILUMINESCENT IMMUNOMETRIC ASSAY

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Saakshi

**Dr Saakshi Mittal**
**MD(Path)**

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| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| **SERUM ZINC** | **PHOTOMETRY** | **155** | **µg/dL** |

**Bio. Ref. Interval. :-**

70 - 115

Clinical Significance:
Zinc is one of the essential trace elements in the body. Its metalloenzymes play a key role in protein and nucleic acid synthesis, gene expression, wound healing, as an antioxidant, etc. Deficiency can cause- Poor wound healing, gastroenteritis, impaired spermatogenesis, Alzheimer's disease, etc. Toxicity may be manifested as pancreatitis, gastric ulcer, anemia, pulmonary fibrosis.

Specifications:
Precision: Intra assay (%CV): 2.02, Inter assay (%CV): 2.22.

Kit Validation References:
Thomas L. Clinical Laboratory Diagnostics. 1st ed. Frankfurt: TH-Books Verlagsgesellschaft; 1998. p. 347-9

**Please correlate with clinical conditions.**
**Method:-** NITRO - PAPS

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

**Dr.Vinitha Jose**
**MD(Path)**

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MC-5360

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| | | **Sample Type \| Barcode** | : **SERUM \| FN883083** |

| TEST NAME | TECHNOLOGY | VALUE | UNITS | Bio. Ref. Interval |
|---|---|---|---|---|
| CALCIUM | PHOTOMETRY | 9.29 | mg/dL | 8.8-10.6 |
| PHOSPHOROUS | PHOTOMETRY | 4.85 | mg/dL | 2.4 - 5.1 |

**Please correlate with clinical conditions.**

**Method :**

CALC - Arsenazo III Method, End Point.
PHOS - UNREDUCED PHOSPHOMOLYBDATE METHOD

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| | | **Sample Type \| Barcode** | : **SERUM \| FN883083** |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| MAGNESIUM | PHOTOMETRY | 2.4 | mg/dL |

**Bio. Ref. Interval. :-**

1.90 - 3.10 mg/dL

Clinical significance:

Magnesium is the fourth most abundant cation in the body and second most prevalent intracellular cation. The total body magnesium content is about 25 g or approximately 1 mol, of which 55% reside in the skeleton. About 45% of the magnesium is intracellular. In general higher the metabolic activity of cell, the greater is its magnesium content. Magnesium is a cofactor for more than 300 enzymes in the body.

Disorders of magnesium metabolism are separated into those causing hypomagnesaemia/magnesium deficiencies and hypermagnesemia. Hypomagnesaemia is common in patient in hospitals. Moderate to severe deficiency of magnesium is usually due to loss of magnesium from the gastrointestinal (gi) tract or kidneys. One of the more serious complications of magnesium deficiency is cardiac arrhythmia. Symptomatic hypermagnsemia is almost always caused by excessive intake, resulting from administration of antacids, enemas, and parenteral fluids containing magnesium. Depression of neuromuscular system is the most common manifestation of magnesium intoxication.

External quality control program participation:

College Of American Pathologists: Chemistry survey; CAP Number: 7193855-01

**Please correlate with clinical conditions.**

**Method:-** MODIFIED XYLIDYL BLUE REACTION METHOD

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

**Dr.Vinitha Jose**
**MD(Path)**

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Plot No.428,Phase-IV, Udyog
Vihar, Gurgaon,Haryana - 122 015

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| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| DIHYDROTESTOSTERONE (DHT) | E.L.I.S.A | 316.24 | pg/mL |

**Bio. Ref. Interval. :-**

Male:

(1-9 yrs): 0 - 85.7 pg/mL || (10-14 yrs): 11.1 - 875.6 pg/mL
(15-18 yrs): 70.3 - 1260.9 pg/mL || (19-89 yrs): 143 - 842 pg/mL

Female:

(2-9 yrs): 0 - 88.9 pg/mL || (10-14 yrs): 22.5- 280.6 pg/mL
(15-18 yrs): 62.6- 760.3 pg/mL || (18-50 yrs): 0 - 596 pg/mL
(51-83 yrs): 0 - 431 pg/mL

Clinical Significance:

5a-dihydrotestosterone is steroid similar to testosterone and androstenedione. Some of the main clinical indications of the DHT measurement in serum are investigations of Delayed puberty in men and evaluation of the presence of active testicular tissues
Women with too much Dihydrotestosterone may develop increased body, facial and pubic hair growth (called hirsutism), stopping of menstrual periods (amenorrhoea),increased acne and abnormal changes to the genitalia.
Clinical Trends :
1. In Klinefelter's syndrome the DHT level is much more lower than that found in normal men.
2. In polycystic ovaries (PCO) about 35 % of the patients have an increased DHT level.
3. The DHT level in young is much higher than those found in normal older people, hence androgen production increases at puberty which gives rise to masculinizing characteristic.
4. There is very low level of Plasma DHT in patients with anorchia.

**Please correlate with clinical conditions.**
**Method:-** COMPETITIVE ENZYME IMMUNOASSAY

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Saakshi

Dr Saakshi Mittal
MD(Path)

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| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| C-REACTIVE PROTEIN (CRP) | IMMUNOTURBIDIMETRY | 0.17 | mg/L |

**Bio. Ref. Interval. :-**

Acute phase determination : < 5 mg/L

Clinical Significance:
It's a protein present in the sera of acutely ill patients that bound cell wall C-polysaccharide of streptococcus pneumoniae and agglutinates the organisms. CRP is one of the strongest acute -phase reactants, with plasma concentrations rising up after myocardial infarction, stress, trauma, infection, inflammation, surgery, or neoplastic proliferation.

Concentrations >5 mg/L suggest the presence of an infection or inflammatory process. Concentrations are generally higher in bacterial than viral infection. The increase in peak is proportional to tissue damage. Determination of CRP is clinically useful to screen activity of inflammatory diseases such as rheumatoid arthritis; SLE;Leukemia;after surgery;to detect rejection in renal allograft recipients;to detect neonatal septicemia and meningitis. However, it is a nonspecific marker and cannot be interpreted without other clinical information.

Reference:
Tietz Textbook of clinical chemistry and molecular diagnosis fifth edition chapter 21 P538-539

**Please correlate with clinical conditions.**

**Method:-** FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

**Dr.Vinitha Jose**
**MD(Path)**

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| | | **Sample Type \| Barcode** | : SERUM \| FN883083 |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| CORTISOL | C.L.I.A | 8.32 | µg/dL |

**Bio. Ref. Interval. :-**

6.00am to 10am: 6.7 - 22.6 µg/dL
4.00pm to 8pm: <10.00 µg/dL

Clinical Significance:

- Cortisol is the major glucocorticoid product of the adrenal cortex.
- Cortisol plays an important role in the regulation of many physiological processes like energy metabolism, maintaining electrolyte balance and blood pressure, immunomodulation and stress responses, cell proliferation as well as cognitive function.
- Elevated serum levels of cortisol can also be seen in stress responses, psychiatric diseases, obesity, diabetes, alcoholism, contraceptives, estrogen therapy and pregnancy which may cause diagnostic problems in patients with Cushings Syndrome.
- Cortisol secretion is controlled by Adrenocorticotropic hormone (ACTH) of pituitary gland
- Normally, the Cortisol secretion happens in the second half of the night with peak cortisol production occurring in early morning, cortisol levels then decline throughout the day with lowest levels during first half of the night.
- Therefore circadian variations of cortisol release and influence of stress should be considered while sampling for cortisol test
- Samples should not be taken from, patients receiving biotin therapy, until at least 8 hours following the last biotin administration
- In some cases, interference is seen due to presence of antibodies to analyte-specific antibodies, streptavidin or ruthenium (in case of ECLIA method) can occur and interference with heterophile and HAMA antibodies can occur (in case of CLIA method assay)
- Cortisol is best measured in the morning when evaluating for possible Adrenal Insufficiency and best measured in the afternoon or evening to differentiate normal and Cushings Syndrome subjects.
- Increased Cortisol levels seen in pathologies like : Cushings Syndrome, Ectopic ACTH syndrome, prednisolone therapy, Ectopic CRH syndrome, Adrenal adenoma / carcinoma, Adrenal micronodular dysplasia, Adrenal macronodular hyperplasia, Stress
- Decreased Cortisol Levels seen in pathologies like - Addisons disease, Pituitary dysfunction
- The results should be interpreted in conjunction with clinical history and other findings

References: Kit insert

**Please correlate with clinical conditions.**

**Method:-** Competitive binding Immunoenzymatic assay

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Saakshi

**Dr Saakshi Mittal**
**MD(Path)**

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| | | | |
|---|---|---|---|
| **Patient Name** | : **MR SAM DANIEL (24Y/M)** | **Sample Collected on (SCT)** | : **29 Sep 2026 10:57** |
| **Referred By** | : **SELF** | **Sample Received on (SRT)** | : **29 Sep 2026 12:22** |
| **Address** | : INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007 | **Report Released on (RRT)** | : **30 Sep 2026 13:43** |
| | | **Sample Type \| Barcode** | : **SERUM \| FN883083** |

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|---|---|---|---|
| ANTI NUCLEAR ANTIBODIES (ANA) | E.L.I.S.A | 0.09 | OD Ratio |

**Bio. Ref. Interval. :**

Negative: <0.90
Equivocal : 0.90 - 1.10
Positive: >1.11

Clinical Significance:
Anti-nuclear antibodies (ANA) are autoantibodies directed against nuclear components and are commonly associated with autoimmune diseases such as systemic lupus erythematosus (SLE), mixed connective tissue disease (MCTD), systemic sclerosis, Sjögren's syndrome, and rheumatoid arthritis.

However, ANA positivity is not specific to any one condition and may also be seen in healthy individuals, especially the elderly, or in cases of infections, malignancies, or drug-induced autoimmunity.

Equivocal results may require repeat testing or additional evaluation with specific antibody panels (e.g., anti-Sm, anti-RNP, anti-Ro, anti-La) for diagnostic clarification. A negative ANA test does not completely rule out autoimmune disease, particularly in early stages or when non-ANA-specific autoantibodies are involved. The final diagnosis should be made by the physician based on the overall clinical picture and appropriate correlation with additional investigations.

Note: Hemolysis, lipemia, icterus, microbial contamination, high-dose biotin, or heterophile antibodies may interfere with test accuracy and may lead to false-positive or false-negative results.

**Method :** INDIRECT SOLID PHASE IMMUNOASSAY

**Please correlate with clinical conditions.**

**Tests Done :** JAANCH - MENS HAIRFALL SCREENING ADVANCED

**Report Remarks :** LabCode:2909041103/FE254

Dr.Vinitha Jose
MD(Path)

Scan QR to verify(valid for 30 days from release time)

## CUSTOMER DETAILS

**As declared in our data base**

**Name:** MR SAM DANIEL **Age:** 24Y **Sex:** M **Customer Mobile No:** 9894377521

| | | |
|---|---|---|
| **Barcodes/Sample_Type** | : | FN883083(SERUM), 525452DO(EDTA) |
| **Labcode** | : | 2909041103, 0210162289 |
| **Ref By** | : | SELF |
| **Sample_Type/Tests** | : | SERUM:JAANCH - MENS HAIRFALL SCREENING ADVANCED<br>EDTA:HEMOGRAM - 6 PART (DIFF) |
| **Address** | : | INGENX LABS Pvt Ltd., P18, Shifa Golden City, Tirunelveli. - 627007 |
| **Sample Collected on (SCT)** | : | 29 Sep 2026 10:57 |
| **Report Released on (RRT)** | : | 30 Sep 2026 13:43 |
| **Amount Collected** | : | **Rs.3495/-(three thousand four hundred and ninety-five only)** |

**Thyrocare,D-37/1,MIDC,Turbhe,Navi Mumbai - 400703. | Phone:022 - 6712 3400 |www.thyrocare.com | info@thyrocare.com**

## CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient, as the reported results are dependent on individual assay method(s) used/ equipments used and quality of specimen received.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume:
  (a) any liability including any bodily injury, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report,
  (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link:- https://youtu.be/nbdYeRgYyQc

## EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

## SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at customersupport@thyrocare.com or call us on 022-3090 0000.


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*As per survey on doctors' perception of laboratory diagnostics (IJARIIT, 2023), -Mumbai Reference Lab is CAP Accredited